



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°125/MVEBDB/16/09/2026

Report date: 16 September 2026

Publication date: September 17, 2026



HIGHLIGHTS (last 24 hours)

A total of 71 new confirmed cases, including 18 community deaths from Ebola Virus Disease (EVD), have been reported during the day of September 16, 2026, particularly in the provinces of Ituri (41 cases), North Kivu (27 cases), Haut-Uélé (2) and Tshopo (1 case). Furthermore, 10 deaths within the Ebola Treatment Center (ETC) were recorded.

No new health zones were affected during the day of September 16, 2026.

ACCUMULATIVE		ACCUMULATIVE		PATIENTS IN	ACCUMULATIVE	TRACKING RATES
CAS		DEATH		ISOLATION/CTE	HEALED	CONTACTS
7 475		CONFIRMED		905	1 798	(Of the Day)
		3 605				87,6%
			LETHALITY			
			48,2%			



Affected Provinces and Affected Health Zones



7 Provinces affected

Uele, Ituri, North Kivu,
South Kivu, Tshopo, Lower Uele and
Sud Ubangi



62 out of 167
health zones affected
(37,1%)

Summary of key points from the last 24 hours

At the end of the day on September 16, 2026, **71 new confirmed cases** of Ebola Virus Disease (EVD) were reported, including **18 community deaths**.

The provinces that reported these confirmed cases are **Ituri (41 cases)**, **North Kivu (27 cases)**, **Haut-Uélé (2 cases)** and **Tshopo (1 case)**, while Bas-Uélé, South Kivu and South Ubangi did not record any new confirmed cases.

Meanwhile, **10 intra-CTE deaths** (9 in Ituri and 1 in North Kivu) were recorded during the same day, bringing the total number of deaths for the day to **28**.

On the other hand, **22 patients** were declared cured (**18 in Ituri**, **2 in Haut-Uélé** and **2 in Nord-Kivu**) after obtaining two negative results for Ebola virus disease.

Based on data reported since the start of the epidemic in the DRC, a cumulative total of **7,475 confirmed cases**, including **3,605 deaths**, has been recorded, representing a case fatality rate of **48.2 %**. The epicenter remains Ituri province, which accounts for **77.5% of the cases**. confirmed cases reported in the 7 affected provinces and also holds (**57.7 %**) of new confirmed cases.

No new health zones have been affected in the last 24 hours.

The DRC remains with **62 out of 167 health zones (37.1%)** affected across **7 affected provinces**. Ituri has 28 out of 36 affected health zones, followed by North Kivu (16/34), Haut-Uélé (6/13), Tshopo (7/23), South Kivu (1/34), Bas-Uélé (3/11) and South Ubangi (1/16).

Table 1. Distribution of confirmed cases and deaths by affected province, as of September 16, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	41	5792	2 642	45,6%	28/36 (77,8 %)
North Kivu	27	1350	828	61,3%	16/34 (47,1 %)
Haut-Uélé	2	292	118	40,4%	6/13 (46,2 %)
Boss	1	30	11	36,7%	7/23 (30,4 %)
South Kivu	0	3	1	33,3%	1/34 (2,9 %)
Bas-Uélé	0	7	4	57,1%	3/11 (27,3%)
Sud Ubangi	0	1	1	100,0%	1/16 (6,3%)
Total	71	7 475	3 605	48,2%	62/167 (37,1 %)

Table 2. Distribution of confirmed cases and deaths by province and zone as of September 16, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cas (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	5792	2642	45,6%	41	11	9	20
Give	11	1	9,1%				0
Underwood	8	6	75,0%				0
Ariwara	8	2	25,0%				0
Aungba	12	5	41,7%				0
Bamboo	183	39	21,3%	4	0		0
have	2	2	100,0%				0
Bunia	1620	511	31,5%	9	1		1
Ladies	40	9	22,5%				0
Drodre	15	7	46,7%				0
Fireworks	80	30	37,5%	1	0		0
Gethy	7	3	42,9%				0
Flounder	2	0	0,0%				0
Kilo	34	12	35,3%	1	0		0
Team	138	88	63,8%	2	0		0
Liter	267	132	49,4%	3	2		2
Logo	13	5	38,5%				0
Fight	31	8	25,8%				0
distributed	4	1	25,0%				0
Washing	21	9	42,9%	4	2		2
Darkness	59	33	55,9%	2	1		1
Mangala	326	146	44,8%	4	0		0
Mongbwalu	672	308	45,8%	2	1		1
Nanny	253	132	52,2%	5	4		4
Nizi	743	273	36,7%	1	0		0
Legumes	125	36	28,8%				0
Jungle	11	4	36,4%				0

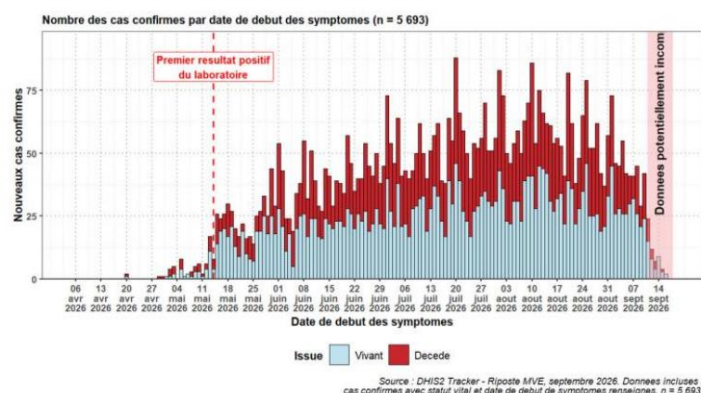
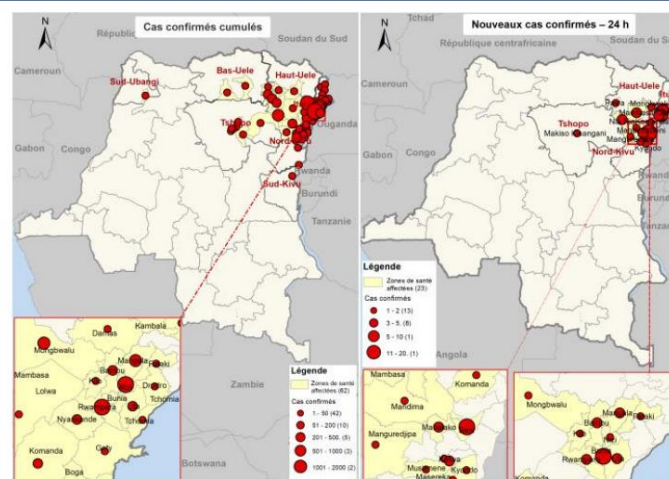
Rwampara	1032	350	33,9%	3	0		0
Tchomia	75	25	33,3%				0
A valves	THAT	465	THAT			9	9
North Kivu	1350	828	61,3%	27	6	1	7
Me	255	183	71,8%	14	5		5
Farm	18	7	38,9%				0
Butembo	244	157	64,3%	2	0		0
Ten	1	0	0,0%				0
Kalunguta	36	12	33,3%				0
Cut	536	339	63,2%	3	1	1	2
Source	1	0	0,0%				0
Kyondo	32	20	62,5%	2	0		0
Lubero	15	7	46,7%				0
Be careful.	22	9	40,9%	3	0		0
Manguredjipa	10	3	30,0%	1	0		0
Masereka	24	9	37,5%	1	0		0
The museums	118	64	54,2%	1	0		0
Mutwanga	6	2	33,3%				0
Oicha	12	10	83,3%				0
Vuh	20	6	30,0%				0
Haut-Uélé	292	118	40,4%	2	0	0	0
Kill Mangbetu	34	15	44,1%				0
Gombari	3	1	33,3%				0
Mathematics	98	42	42,9%				0
Lining	63	26	41,3%	2	0		0
Beans	2	1	50,0%				0
Ordinary	92	33	35,9%				0
Boss	30	11	36,7%	1	1	0	1
Bafwasende	7	1	14,3%				0
Kabondo	4	1	25,0%				0
Lubunga	1	0	0,0%				0
Maquis-Kisangani	12	6	50,0%	1	1	0	1
Mangobo	4	2	50,0%				0
Boss	1	0	0,0%				0
Wanie-Rukula	1	1	100,0%				0
South Kivu	3	1	33,3%	0	0	0	0
Mili-Murhesa	3	1	33,3%				0
Bas-Uélé	7	4	57,1%	0	0	0	0
Blind	1	1	100,0%				0
Viadana	3	1	33,3%				0
Walk	3	2	66,7%				0
Sud Ubangi	1	1	100,0%	0	0	0	0
Hair	1	1	100,0%				0
Total	7475	3605	48,2%	71	18	10	28

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA : Not Applicable.

Table 3. Status of alerts notified by province for September 16, 2026

Table of Statistics of alerts notified by province for September 20, 2020									
DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	12	8	20	5	8	5	0	13	4
Bas-Uélé	62	1	63	4	1	58	0	5	0
Ituri	793	72	865	106	67	506	5	173	95
North Kivu	1 112	57	1169	160	57	836	0	217	101
South Kivu	9	0	9	2	0	6	0	2	0
Sud Ubangi			0						
Boss	94	2	96	7	0	87	2	7	2
Total General	2 082	140	2 222	284	133	1 498	7	417	202

Figure 1. Evolution of cases by date of symptom onset over the last 21 days**Figure 2. Spatial distribution of confirmed cases (24h and cumulative) by health zone.**

1. COORDINATION

1.1. Main Actions

- Monitoring and support of response actions in all affected provinces.

2. RESPONSE ACTIONS BY PILLAR

1.1.1. Epidemiological surveillance

1.1.2. Main Actions

With the exception of South Ubangi province, the six other affected provinces reported a total of 2,222 alerts as of September 16, 2026, of which 1,922 (86.5%) were verified. Thus, 417 alerts were validated as suspected cases. All of these were investigated (100%), and 202 were transferred to treatment centers (Table 3).

Of the 29,873 contacts currently being monitored, 26,181 were seen in the last 24 hours, representing a follow-up rate of 87.6%, roughly the same as the previous day (87.6%). In Bas-Uélé, 172 out of 180 contacts under follow-up were seen, representing 95.6%; 90.7% (13,535/14,926) in Ituri; 85.6% (11,604/13,556) in North Kivu; 82.2% (267/325) in Tshopo; 69.3% (587/847) in Haut-Uélé; and 41.0% (16/39) in South Ubangi.

1.2. Update on checkpoint and point of entry (PoE/PoC) activities

1.2.1. Main actions

Three (3) alerts were notified to the PoE/PoC in the last 24 hours, two (2) in Ituri and one (1) in North Kivu. In Ituri, a live alert was intercepted at the PoC of 4

In Bey (Komanda Health Zone), a case was validated but transfer to the CTE was refused; a second case, presenting signs of hemorrhage, was intercepted at the Rubingo PoC (Boga Health Zone) and then transferred to the Boga General Referral Hospital. In North Kivu, a body was intercepted at the Mavivi PoC (Beni Health Zone) and sent to the morgue for swab testing. In Bas-Uélé, two PoCs in the Ganga Health Zone were activated.

Table 4. Status of key indicators for PoE/PoC by province, as of September 16, 2026

Indicators	Ituri	North Kivu	South Kivu	Boss	Haut-Uélé	Global
Report completeness	98,3% (59/60) 100%	(20/20) 55,6% (5/9)	100% (18/18) 100%	(7/7) 95,6% (109/114)		
% of functional strategic PoE/PoC	100%	100%	100%	100%	100%	100%
Number of people who have passed PoE/PoC	140 196	85 252	8 945	13 317	14 825	262 535
% of travelers screened	97,0%	95,8%	98,8%	99,3%	99,0%	97,8%
% of travelers who washed their hands	97,0%	95,8%	98,8%	97,9%	99,0%	97,5%
% of travelers made aware	99,0%	95,8%	100,0%	100,0%	100,0%	98,9%
Number of alerts notified	2	1	0	0	0	3
Number of validated live alerts (cas suspects)	2	0	0	0	0	2
Number of bodies recovered today	0	1	0	0	0	1
% of suspected cases transferred to CT/CTE	50% (1/2)	THAT	THAT	THAT	THAT	50% (1/2)
% of lifeless bodies swabbed/secured	THAT	100% (1/1)	THAT	THAT	THAT	100% (1/1)
% of suspected cases investigated within 24 h	100% (2/2)	THAT	THAT	THAT	THAT	100% (2/2)

1.2.2. Challenges

Suboptimal operationalization of PoE/PoC coupled with low reporting completeness in South Kivu (55.6%, 5/9); persistent insecurity hindering the commissioning of the Avakubi PoC (Ituri), low alert coding in DHIS2 Tracker, and insufficient mobility in Rwampara and Bunia. In North Kivu, lack of security personnel support for PoCs along the Butembo axis and threats against service providers. In South Kivu, absence of isolation measures at the Kavimvira, Kalundu Port SNCC, and Baraka PoEs. In Bas-Uélé, 16 PoCs activated and PoEs reinforced out of 41 (39.0%), and a lack of trained personnel.

1.3. Laboratory

1.3.1. Main actions

- **Ituri : 41 new positive results** (30 living and 11 deaths) out of 204 new samples received and analyzed (**positivity: 20.1%**).
- **North Kivu: 27 samples received and tested** (21 live and 6 dead) out of 217 samples analyzed (**positivity of 12.4%**).
- **Haut-Uélé: 2 new positive results (2 live) out of 16 samples received and analyzed at the Isiro laboratory (positivity: 12.5%).**
- **Tshopo: 1 new positive result** (1 live sample), out of 7 samples received and tested (positivity: 14.3%).

- **Bas-Uélé:** 3 samples received and analyzed (3 live) all returned negative.

1.4. Infection Prevention and Control (IPC/EDS)

1.4.1. Main actions

- **In North Kivu,** 27 of the 33 planned infection control rings were opened (81.8%), 11 of the 36 identified households were decontaminated (30.6%), 34 of the 42 health facilities were decontaminated (81.0%), and 1 of the 11 households received infection prevention and control (IPC) kits (9.1%); 7 scorecard assessments were conducted in 5 health establishments (average score of 23.7%); 658 service providers were briefed, 199 health establishments were monitored, and the use of 339 handwashing stations and 8 triage units was monitored. Regarding the emergency response plans (ERP), 56 alerts were recorded, 51 ERPs were carried out, and 56 bodies were swabbed (100%).
- **In Tshopo,** decontamination of 5 premises, 2 latrines and 2 showers of the CM Ndako ya Elikya around the 29th confirmed case; assessment of exposed PPL (6 at high risk and 11 at low risk at CM Ndako ya Elikya as well as 3 at high risk at CS Bethléem); briefing of 5 hygienists in the AS of Bafwandambo; monitoring and support of 11 ESS in 5 health zones with 135 providers briefed on standard precautions.
- **In Haut-Uélé,** 12 out of 12 planned screenings were opened (100%), 27 out of 35 identified households were decontaminated (77.1%), and all 4 health facilities were decontaminated (100%); 10 community health centers (Isiro 4, Pawa 3, Boma Mangbetu 3) were supervised, with 12 service providers briefed, and the Canana health post was opened. Regarding the health surveillance system, 7 alerts were recorded, 6 surveillance operations were carried out, and 6 bodies were swabbed.
- **In Bas-Uélé,** 2 out of 2 expected infection control zones (100%) were opened in Viadana, but neither of the 2 identified households nor the identified health facility (FOSA) has been decontaminated; briefings were conducted for 17 public health workers and 5 hygiene specialists in 6 health centers (ESS) in the health zones of Ango, Monga, and Ganga on the use of personal protective equipment (PPE), injection safety, and hand hygiene. Regarding the blood and casualty incidents (BSI), 1 alert was recorded and 1 body was swabbed in Ganga, without a BSI being carried out.

1.4.2. Challenges

- **In North Kivu :** low coverage of household decontamination (30.6%; 11/36) and need to activate, train and motivate decontamination teams in the health zones of Biena, Masereka, Kyondo, Mabalako and Vuhovi; lack of traceability of some eligible households and addresses not found for the implementation of community activities; lack of mobility for the deployment of provincial IPC teams; urgent need for training for all IPC Data on data collection and management tools; insufficient handwashing facilities in schools.
- **In Tshopo :** insufficient IPC inputs in households, schools, markets and churches; low proportion of SSE with an IPC score above 80% (2.2%; 2/93); absolute resistance of cases 20 and 26 despite several negotiations to carry out public health actions; incinerator at HGR Makiso-Kisangani still out of order.

1.5. Holistic care

1.5.1. Main actions

- **In Ituri,** there were 89 new admissions to the various dedicated facilities, compared to 107 discharges, including 18 recoveries. Thus, 442 patients are hospitalized out of 1015 beds, representing an occupancy rate of 43.5%; 405/429 (94.4%) patients received hot meals; 10/10 children are receiving LNPE/SLM at the CTE, 4/4 are in the nursery, and 21/21 are in the community; 34 patients are in critical condition, and 3 transfers were organized from the Mangala Treatment Center to the Nizi CTE.
- **In North Kivu,** 103 new admissions were recorded compared to 60 discharges, including 2 recoveries; 376 patients are hospitalized, including 221 in standardized facilities with a capacity of 6

of 228 beds, i.e. an occupancy rate (96.9%), close to saturation; 35/35 patients, 10/10 accompanying persons and 52/52 frontline providers received a hot meal and 9/9 non-breastfed orphans are on breast milk substitute; 8 patients are in critical condition.

- **In Haut-Uélé**, there were 10 new admissions compared to 2 discharges (recovered). Thus, 62 patients are hospitalized out of 136 beds, representing an occupancy rate of 45.6%; 62 patients, 67 accompanying persons and 31 PPLs received three hot meals, except in Rungu and Gombari.
- **In Bas-Uélé**, one (1) new admission against one (1) discharge (1 non-case); 5 patients remain in isolation including 1 confirmed case taken care of at the HGR of Ganga; 9 high risk contacts remain under prophylaxis and 10 have left unaffected from the follow-up.
- **In Tshopo**, one (1) new admission was recorded, a 35-year-old male patient with a history of AIDS, and no discharges. Seven (7) patients are in isolation out of 31 beds, representing an occupancy rate of 22.6%; 4 confirmed and 2 suspected cases received a hot meal, and 42/42 (100%) frontline providers were served.
- **In South Kivu**, 2 new admissions were recorded and 5 discharges were reported. Thirteen (13) Patients are in isolation for 25 beds, representing an occupancy rate of 52.0%.

1.5.2. Challenges

Treatment remains partially substandard and capacity is limited: **In Ituri**, Ebola Treatment Centers (ETCs) are saturated with suspected cases in Nizi, Mambasa, Lolwa, Kilo, Mangala, and Mandima. **In North Kivu**, 42.1% of hospitalized patients are treated outside of standardized facilities; no beds are available in Butembo and Katwa. **In Tshopo**, the lack of safe blood and limited technical resources persist. **In Bas-Uélé**, the absence of ETCs and ambulances hinders the isolation of cases.

1.6. Vaccination

1.6.1. Main actions

- **In Tshopo**, 2,810 people have been vaccinated to date out of 11,703 targeted TPL/PPL (24.0%), including 1,633 in Makiso-Kisangani, 381 in Kabondo, 352 in Mangobo, 280 in Tshopo, 126 in Lubunga and 38 in Bafwasende; the remaining 500 frozen doses are available.
- **In Bas-Uélé**, 600 people have been vaccinated to date, including 550 in Buta and 50 in Ganga; compassionate vaccination has begun in Ganga with the 450 available doses, and the transport of cold chain equipment to the EPI (Expanded Program on Immunization) unit in Buta is ongoing. In Haut-Uélé, cold chain equipment for vaccines has been received.
- **In Ituri**, preparations for the vaccination of PPL continue.

1.6.2. Challenges

Low uptake of PPL and TPL in Tshopo, with a reported refusal in the Mandombe health district and the cancellation of the September 16, 2026 sessions in Lubunga, Mangobo, and Kabondo due to lack of mobility; the Bafwasende site remains underperforming (4% of the target population); vaccination rate at the Bafwasende site remains low (4% of the target population). (affected). In Bas-Uélé, there is no microplan for the vaccination of PPL in the six priority health zones (Aketi, Ango, Bili, Buta, Ganga and Viadana) and additional doses of ERVEBO are needed.

1.7. Continuity of Care

- **In Tshopo**, 139 cases received the experimental treatment, including 19 confirmed cases and 127 under post-exposure prophylaxis (87 completed, 11 interrupted, and 29 ongoing); evaluation of the Bethlehem health post (Mangobo), development of the flat-rate pricing document, receipt of medications from the UG-PDSS project, and monitoring of PPLs. A stockout of HIV RDTs at the PNLS central office and low usage of handwashing facilities were noted.

- In **Haut-Uélé**, continued briefing of service providers on the specific and prophylactic treatment of Ebola virus disease and preparation of lists of recovered patients awaiting discharge as well as lists of PPLs according to their status.
- In **Bas-Uélé**, follow-up of contacts who have adhered to compassionate prophylaxis in Buta and Dingila and post-exposure prophylaxis of PPL high-risk contacts in the Ganga health zone.

1.8. Risk communication and community engagement

1.8.1. Main actions

- In **Ituri**, 18,121 home visits reached 56,340 people and 1,601 educational talks reached 24,519 people; 8 mass awareness campaigns reached 901 people, 36 community debates reached 584 and 5 advocacy campaigns reached 6; 227 community alerts were notified (including 40 deaths), 40 of the 61 feedbacks and 38 of the 55 incidents were resolved within 48 hours (completeness: 12/28 health zones; 43%); support was provided for the pre-listing of 222 contacts, the decontamination of 30 households, 30 of the 33 EDS (90.9%) and the transfer of 28 suspected cases.
- In **North Kivu**, 22,382 people were reached during 7,493 home visits, 2,783 during 63 educational talks and 3,250 during 61 mass communication actions; 157 community alerts notified out of 450 expected (34.9%), 8 feedbacks resolved out of 13 (61.5%), none of the 3 incidents resolved and 1,452 people engaged out of 4,750 (30.6%); press briefing on Ebola in Butembo with 20 media and first meeting of the CREC pillar of the Mutwanga Health Zone.
- In **Haut-Uélé**, 9 community engagement activities reached 1,361 people, including 2 mass activities (1,160), 3 educational talks (95), 3 home visits (88) and 1 dialogue (18); 8 alerts of suspected cases and 7 of community deaths notified, 3 feedbacks resolved out of 160 (1.9%) and 1,388 people engaged; a broadcast on RTNC/Isiro denied the rumor of vaccination of students and actions around the last 9 cases reached 105 people.

In Bas-Uélé, 500 people were sensitized in schools (350 in Buta, 150 in Poko), 106 food vendors in Bondo and 3 advocacy efforts were undertaken with the heads of schools in Buta.

- In **Tshopo**, 732 households visited at home reached 1,099 people and 2 educational talks reached 89 participants in Bafwandambo and Baego; 3 mass awareness campaigns reached 2,760 people in Kabondo and Tshopo, 4 interactive programs and 16 spots were broadcast in 8 media outlets and 175 students and teachers were sensitized; 15 community alerts were notified and all resolved, as well as one feedback issue resolved within 48 hours.

1.8.2. Challenges

Persistent community resistance: In **Tshopo**, a confirmed case who escaped from the Ebola Treatment Center (ETC) remains at home, and refusals to be included in the list are reported around the 29th case. In Ituri, data collection is low in some health zones, and demotivation among stakeholders is linked to unpaid bills. In North Kivu, a pressure group is threatening the Vunyakondomi Regional Emergency Coordination Centers (RECOs), and there is dissatisfaction with the care provided at the ETC. In Haut-Uélé, visibility tools have broken down in all affected health zones. In Bas-Uélé, community alerts are being reported infrequently.

1.9. Mental Health and Psychosocial Support (MHPSS)

1.9.1. Main actions

- In **Ituri**, 20 people among the new confirmed cases and 82 of the 145 confirmed cases under follow-up (56.6%) benefited from SMSPS interventions, as well as 21 of the 64 new suspects (32.8%) and 72 of the 104 suspects under follow-up; 44 results announced, including 20 positive cases, 218 caregivers and 74 PPLs supported, 7/7 children followed in daycare, 12 of the 18 households of separated children accompanied, 22/22 recovered individuals followed in the community and 20 recovered individuals reintegrated; 6 of the 9 notified health zones are covered, for 10 of the 28 zones in the province.
- In **North Kivu**, 21 people among the newly confirmed cases and 201 confirmed cases under monitoring were treated, as well as 131 new suspected cases and 246 suspected cases under monitoring; 133 results were announced, including 21 positive cases, 57 people receiving support, 66 households receiving assistance, and 21/21 recoveries followed up in the community; 37/37 children are being followed up in the community, 3/3 are in daycare, and 30 of the 36 households with separated children are receiving assistance (83.3%); 443 people benefited from 8

psychoeducation with the support of UNICEF and the 6 notified health zones are covered.

- **In Tshopo**, 1 out of 1 confirmed case is being monitored, 3 of the 3 confirmed cases being monitored have benefited from SMSPS interventions, as well as one suspected case being monitored; 2 results have been announced, including 1 positive case and 1 household supported; 2 of the 4 notified health zones are covered.

In Haut -**Uélé**, 2 new confirmed cases and 19 confirmed cases under observation received SMSPS interventions, as well as 6 new suspected cases and 6 suspected cases under observation; 15 results were announced, including 3 positive cases; 71 caregivers and 38 people with disabilities received support; 5 separated children received community support; and 11 household members received support. A 3-year-old separated child tested positive in Wamba, and 46 recovered individuals are awaiting their discharge kits. In Bas-Uélé, support for the confirmed case in Ganga remains limited by the lack of a psychologist.

1.9.2. Challenges

- **In Ituri**, low coverage of SMSPS interventions (10 of the 28 health zones covered). **In North Kivu**, Low coverage around the latest confirmed cases in nine health zones, lack of childcare facilities in 10 affected areas, and insufficient human resources. **In Tshopo**, there is no SMSPS (Social and Medical Prevention Service) provider in Wanie-Rukula and Bafwasende. **In Haut-Uélé**, there are difficulties with the mobility of community health workers in Wamba and Boma Mangbetu, a lack of school and nutritional kits for separated children, and poor data reporting in Boma Mangbetu and Pawa. In Bas-Uélé, there is no psychologist in the Ganga health zone.

1.10. Logistics

1.10.1. Main actions

- **In North Kivu**: 11 ambulances available, of which 7 were used (64%), for 18 calls received and 18 missions completed, all successful (100%), with 80% data completeness. • **In Tshopo** : monitoring of the construction of the Madula isolation ward and the Bafwasende Ebola Treatment Center, support for the mobility of the IPC, PECH, communication, PoE/PoC, and vaccination pillars, updating of stocks, and receipt of needs assessments; 3 ambulances available, of which 1 was used (33.3%), for 2 missions, 1 of which was successful. Challenges include a shortage of 154 motorcycles, stockouts of certain supplies, and the need for a second vehicle for Makiso.
- **In Haut-Uélé**: unpacking of a batch of supplies received from UNICEF and WHO; delivery of IPC supplies to the health zones of Boma-Mangbetu, Pawa, and Viadana; mobility support with 5 vehicles and fuel for the ambulance at the Isiro General Referral Hospital (balance 460 L); work at the Isiro Ebola Treatment Center remains suspended. In Bas-Uélé: mobility ensured for the Ganga rapid response team and the alert verification teams, fuel for the laboratory, and a donation of FHI 360 projection equipment.

1.10.2. Security

1.10.3. Main actions

- **In North Kivu**, the pillar provided support and security coverage for IPC teams for the decontamination of households in the health districts of Mangote, Benegule, Kanzabi and Tuungane as well as the CH Sifa, CM La Brèche and CS La Charité facilities; the EDS teams were accompanied for the burial of confirmed cases at the family cemeteries of Mukulya and Bundji and for the swab of lifeless bodies at CH Maombi and Rughenda, with 100% coverage of the interventions requested.

1.10.4. Challenges

In North Kivu, there is a lack of a permanent vehicle for the security team, a lack of financial incentives for agents, and non-payment of police officers and security agents assigned to guard the ESS; there is also a lack of support from security elements in several PoCs along the Butembo axis.

(Kangote, Kyaghala, Mususa and Komba); delayed payment of response actors and failure to meet commitments by some partners in several health zones.

1.11. PSEA (Prevention of Sexual Exploitation and Abuse)

1.11.1. Main actions

- **In Ituri**, 16 staff members received the PSEA briefing (7 men and 9 women) and 217 people were sensitized in the community (77 men and 140 women) in the health zones of Bunia, Rwampara and Gety; 3 partners (WHO, IMC and Save the Children) report their PSEA activities.
- **Haut-Uélé**: 4 of the 6 health zones have carried out PSEA activities: 40 people sensitized in Isiro, 26 PRESICODESA in Gombari, 136 heads of schools in Wamba and 6 people in Pawa, 40 codes of good conduct having been signed; the 6 health zones know the reporting mechanism (100%).

1.11.2. Challenges

- Still insufficient coverage of PSEA activities, with only 13 of the 36 health zones in Ituri (36.1%) aware of the reporting mechanism; lack of mapping of stakeholders and refusal of some partners to align; insufficient mass awareness materials; lack of computer kits and office supplies for the pillar.

MESSAGES CLES

- In the provinces of Ituri, North Kivu and Haut-Uélé, transmission remains high while the
The provinces of Tshopo and Bas Uélé are reporting sporadically.
- In the last 24 hours, **71 new confirmed cases**, including **18 community deaths and 10 intra-CTE deaths**, have been reported, bringing the national total to **7,475 confirmed cases and 3,605 deaths (case fatality rate: 48.2%)**.
- Ituri accounts for **77.5%** of confirmed cases and **57.7%** of new cases reported in the last 24 hours, making it the epicenter of the epidemic.
- The performance of epidemiological investigations remains satisfactory, with 100% of suspected cases investigated and a stable contact tracing rate of **87.6%**.
- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to quickly interrupt transmission chains: Rapidly increase reception capacities in CT/CTE in North Kivu.
- Strengthen preparedness and response to Ebola Virus Disease-B in the provinces of the River Corridor and surrounding areas.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



TASK FORCE PRESIDENTIAL EBOLA 17
Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
MVE17 Incident Management System

Director General of INSP

Dr. MWAMBA KAZADI Dieudonné

Email: dieudonne.mwamba@insp.cd

Incident Manager

Professor Dr. AHUKA MUNDEKE Steve

Email: steve.ahuka@unikin.ac.cd

Operations Manager

Dr. KOBBE NGWAMA John

Editorial Team

Professor Dr. MULANGU KATULUMBA Félix-Médard

Dr. KATUSI BISELENGE Jules

Mrs. WISE DIE Marie

Dr. DIUR MANANASHI Floryan

Mr. Kamuina Kebela John

Contact :

www.insp.cd

contact@sgiebola17.org

Freephone number: 151. www.sante.gouv.cd